Berkeley Haas



MBA297A-HC in 21st Century: Connect Money Stack to Care Journey w/Reimbursement

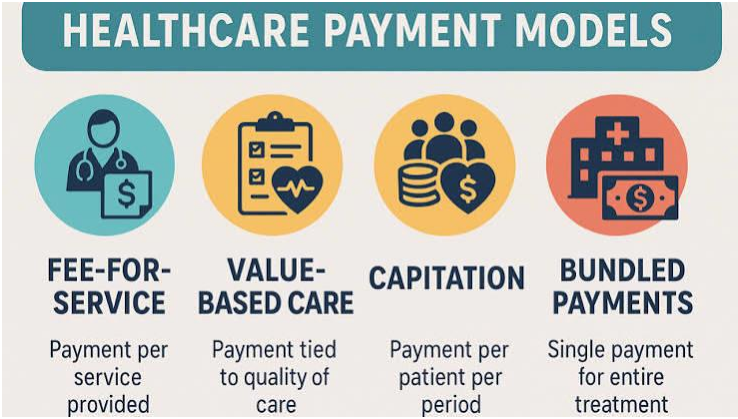
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Flow for this session

- Overview of reimbursement models used by sponsors/ intermediaries for products & care providers
  - Introduce key points along the **Care Journey**
  - How policy drives public & private reimbursement levels/models
- Examine why the transition to value/outcome & risk-based models has been so slow
- 1st In Class Exercise (~12:15pm)





Revisit: Dimensions Often Used to Understand & Compare Health Systems

- Role of government
- Who is covered and how is care financed?
- What is covered (services, products) and under what conditions. **Where is the NO?**
- How is the delivery system organized and financed/paid?
- How are **costs contained**? Is care affordable?
- How important is **equity, access**? How try to achieve?
- How are **new technologies** deployed, used?
- How is quality measured, promoted?
- What are the system's **outcomes** in terms of health, access, equity, cost, efficiency, innovation, etc?

We use payment to influence behavior in the health system – explicit signal of priorities, goals, values

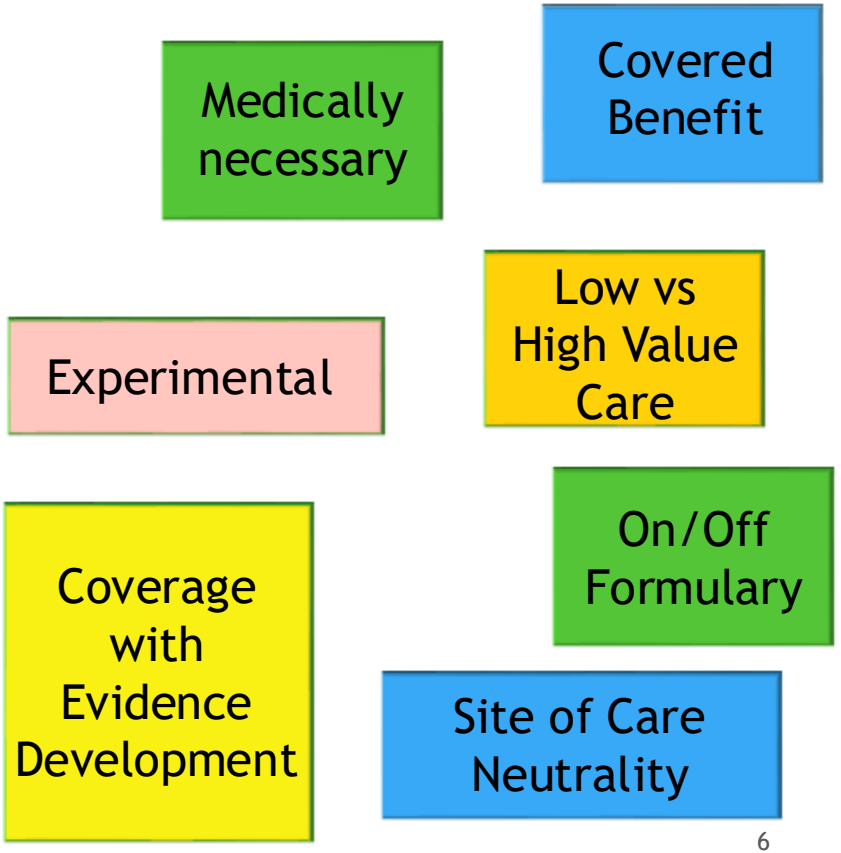
- How something is paid for can be as important as the level of payment (i.e. per service/dose, per episode/case, per 100K people)
  - Prospective vs. retrospective vs. concurrent
  - Certain $$ vs. conditional/contingent on performance
  - Different reimbursement levels and models co-exist for same products, services also can vary by site/modality
- People and organizations are very responsive to payment signals
  - Impact can be muddled by other forces, competing goals
  - Needs to be enough payment to get and keep focus
  - Impact of the payment signal can diminish over time or with competing signals
  - Not everyone or every organization will respond in same way – goals, values, risks, motivations, competing priorities can all be very different

Quick Examples To Consider

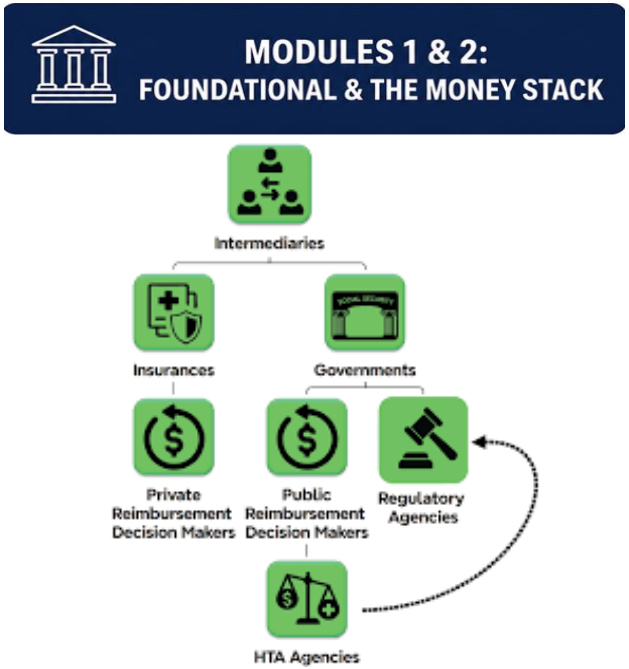
- Will a generic drug manufacturer produce a drug if its reimbursement doesn't cover marginal costs?
- Will MD do all needed screenings, evaluation at primary visit if only paid for a 7-10 minute encounter?
- Will hospitals shift surgeries to the outpatient setting if the reimbursement doesn't cover the higher costs of inpatient care?
- If the payment for submitting quality metrics or encounter data is lower than associated labor costs, will providers collect it?
- Will a digital therapeutic or novel device be available, if there is no existing reimbursement code that covers it?
- Will an expensive oncology treatment be available in a capitated (pre paid budget-based model) given competing needs for the total covered population?

Reimbursement is big hurdle along road to true access of a service or product – who will pay and how much?

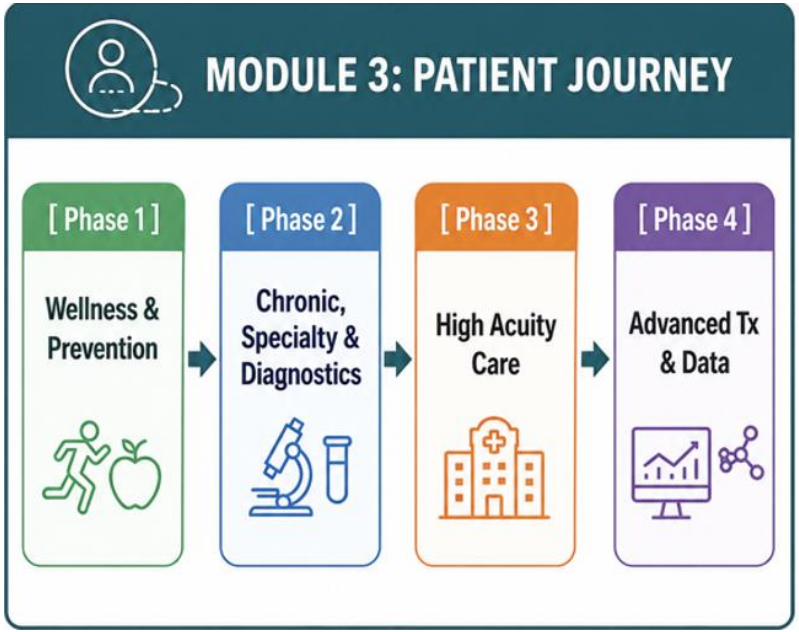
- Just because it exists, doesn't mean a sponsor will pay for it (or for all of it or in all cases)
  - Different issues w/ legacy services and products vs innovative, newer ones
  - Does system already have a way to pay or need new model, code, pathway?
- Capital is essential to develop new products, services, expand health footprint, so investors expect that products/services can generate revenue – key part of business plan



Reimbursement is a clear connector between The Money Stack and Care Journey modules







Volume Based or Fee-for-Service Has Been Dominant Payment Model for Decades

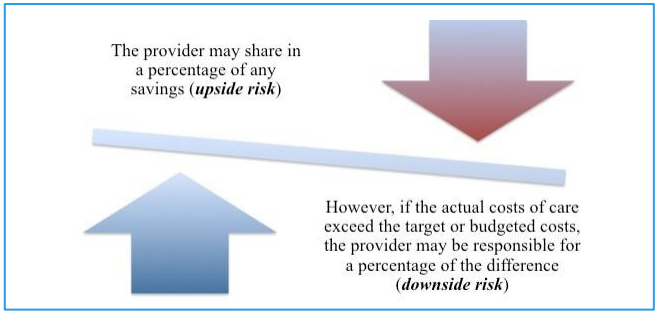
- FFS is a straight-forward way of paying for healthcare that's embedded across the majority of health system processes and data flows – used in US and many other countries
- Called out for being anchored to the **volume of care** and potentially incentivizing more care/product is provided vs an optimal amount- **not linked to outcomes, impact or value**



One major challenge: how define value in way that is actionable, can be used as standard way to pay?

- Outcomes per dollar spent? Which outcome? Who's dollar?
- Therapeutic value of an intervention?
- Value = high quality and safety?
- From an article by Keckly:
  - *In most industries, "value" as defined by consumers is associated with in four attributes:*
    - Accessibility: "can I get what I need or want from you?"
    - Service: "is dealing with you a pleasant experience?"
    - Effectiveness: "is what you're providing going to satisfy my need or want?"
    - Costs: "what's the cost to me and my family and is it worth it?"
- Value in health terms varies by stakeholder, changes over time and can be influenced by the specific situation, emotions and level of urgency – **a moving target, highly dynamic N of 1**

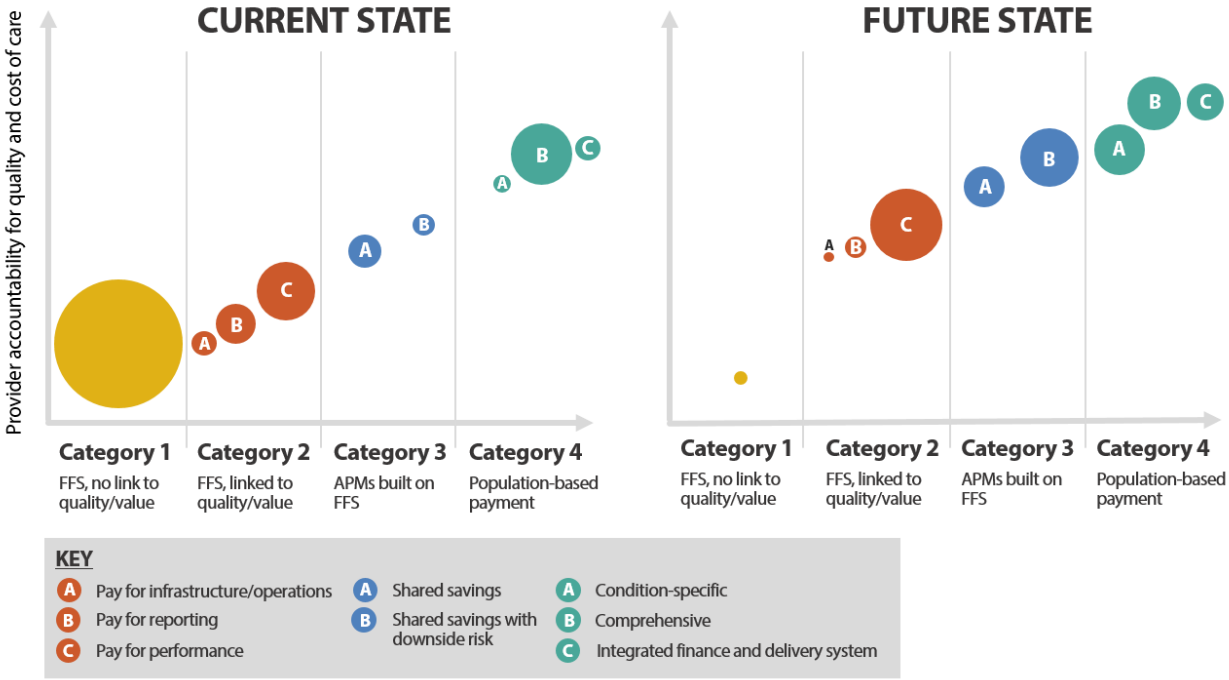
Continuum of Payment Methods from FFS to 2-sided Risk



- Since 2010 been using public policies and private sector initiatives to move along a path to value- based models
  - The ACA created **CMMI to design/pilot new payment models** and the 2012 launch of Medicare Shared Savings Program (MSPP) & ACOs
- Takes major commitment/ investment – need to have **sufficient scale, resources and leadership** focus to make the shift
  - People/population health
  - IT/analytics
  - MD alignment strategies

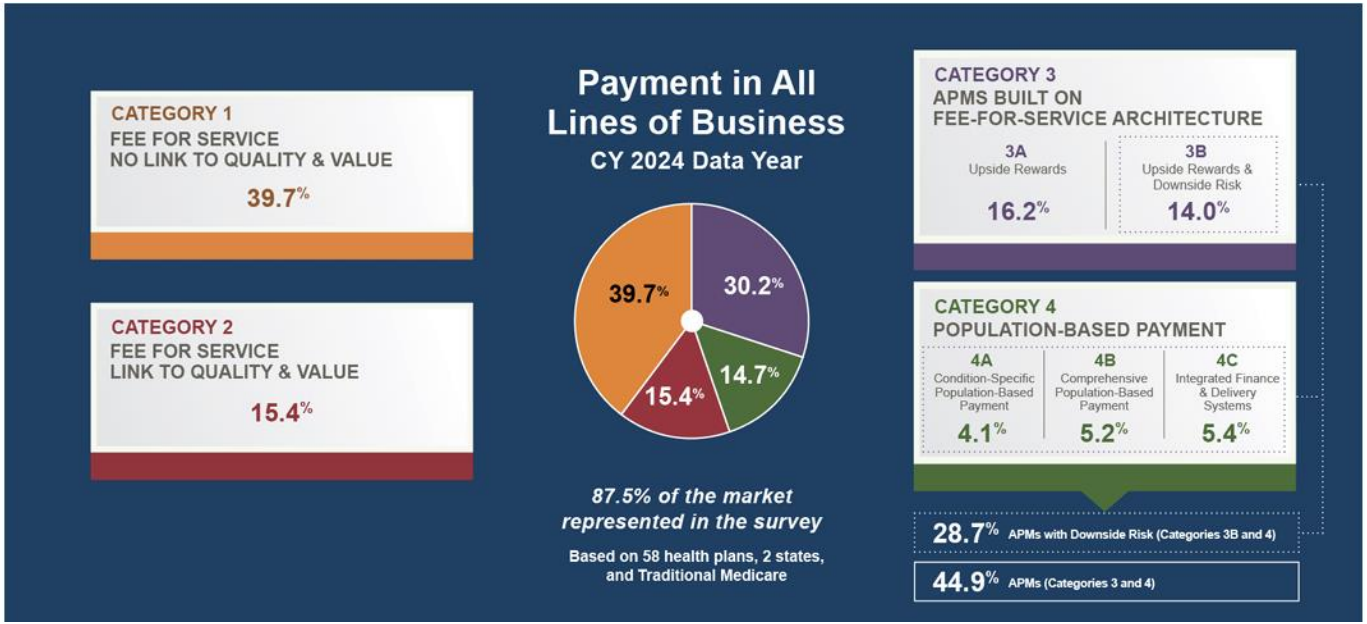
CATEGORY 1	CATEGORY 2	CATEGORY 3	CATEGORY 4
FEE FOR SERVICE – NO LINK TO QUALITY & VALUE	FEE FOR SERVICE – LINK TO QUALITY & VALUE	APMS BUILT ON FEE-FOR-SERVICE ARCHITECTURE	POPULATION – BASED PAYMENT
	A **Foundational Payments for Infrastructure & Operations** (e.g., care coordination fees and payments for HIT investments)	A **APMs with Shared Savings** (e.g., shared savings with upside risk only)	A **Condition-Specific Population-Based Payment** (e.g., per member per month payments, payments for specialty services, such as oncology or mental health)
	B **Pay for Reporting** (e.g., bonuses for reporting data or penalties for not reporting data)	B **APMs with Shared Savings and Downside Risk** (e.g., episode-based payments for procedures and comprehensive payments with upside and downside risk)	B **Comprehensive Population-Based Payment** (e.g., global budgets or full/percent of premium payments)
	C **Pay-for-Performance** (e.g., bonuses for quality performance)		C **Integrated Finance & Delivery System** (e.g., global budgets or full/percent of premium payments in integrated systems)

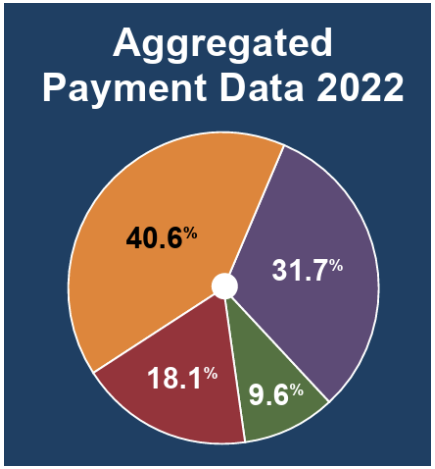
Plan was to move incrementally to advanced forms, increasing accountability for cost and quality



- Goal hasn't been that system would fully move to Category 4
  - Many can't but the goal has been to get away from FFS category
- Huge concern for many with taking on risk for cost and quality and not being able to deliver
  - Reasons/risks often out of their control
  - Harm dynamics with patients
  - Suffer financial losses that would be hard to recover from

Value Based Payment Models Have Spread But Slowly





2024 vs 2022 – FFS is almost the same

If look at data by type of plan, Medicare Advantage has highest Category 4 with 33.4%. Commercial was 7.8% and Medicaid is 9.6%

https://ahiporg-production.s3.amazonaws.com/documents/2025_APM-Methodology_Report.pdf

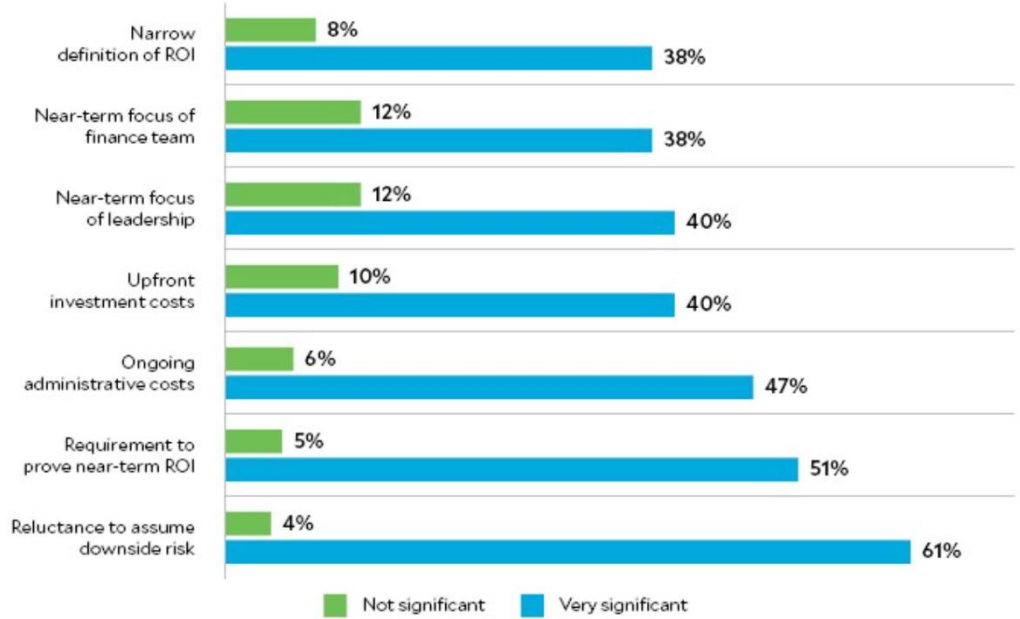
Lagging on Goals – Many Reasons Not Achieving Complete Shift Away from FFS

The goal of the LAN is to accelerate the percentage of U.S. health care payments tied to quality and value in each market segment through adoption of two-sided risk APMs (Categories 3B and 4 of the LAN APM Framework).

	Medicaid	Commercial	Medicare Advantage	Original Medicare
2024	25%	25%	55%	50%
2025	30%	30%	65%	60%
2030	50%	50%	100%	100%

Factors that have played a role in slowing the growth of value-based care

Among the various factors the have contributed to slowing the growth of value-based care in the U.S. healthcare system, reluctance to assume downside risk was cited by 61% of survey respondents as being a very significant factor, while just 4% thought it was not at all significant.



Source: The Terry Group, 2023 perspectives on value-based care, September 2023

Where Does That Leave Things? Straddling Two Canoes

- Most providers along the care journey are doing both to a degree and will be for the foreseeable future – **hybrid systems vs totally abandoning FFS**
- Some external pressures, drivers to accelerate value-based models but no clear, transformational break point with FFS is in sight
- New models and enabling tech solutions keep emerging



Many VBP models come from the government first, advancing policy using its power as a major sponsor

- Range of Advance Payment Method pilots and both optional and mandatory models from CMS/CMMI – focus on various dimensions
  - Point along care journey, provider type, service
  - Incentivizing coordination through Accountable Care Organizational models
  - Condition/disease specific or full continuum
  - Emerging technologies, therapies
- Link to other public policies that support reimbursement mechanics like implementing Electronic Health Records (HITECH Act/Meaningful Use)
- Private sector often will look to adopt next – **welcomes a proven model, already pushed out for mandatory adoption in Medicare, Medicaid**
- **These public policy** driven models impact M&A in legacy companies (vertical integration), investor theses, stimulate new company creation and scale

See New VBP All Along the Care Journey Today

- **Hospitals/Acute Care** – CMS has a new Transforming Episode Accountability Model (TEAM). Runs through 2030, this model makes **downside risk mandatory** for selected acute care hospitals as coordinate major surgical procedures
- **Specialty/Chronic** - Ambulatory Specialty Model (ASM) aims to improve upstream management of chronic disease to reduce avoidable hospitalizations and unnecessary procedures in Medicare. Uses **performance-based payment adjustments for select specialists** who care for beneficiaries with heart failure or low back pain to encourage stronger care coordination and disease management. ASM is a 5 year, mandatory model starting January 1, 2027
- **Primary Care** – Strongest use case with full-risk Medicare Advantage (Oak Street model) but also see big companies like Aledade and Privia Health
- **Newest: Tech enabled care** – CMS has new **ACCESS model** where digital health platforms can act as Medicare Part B providers. Allows them to **manage patient panels asynchronously or virtually and bill** for Outcome-Aligned Payments

Coming up next

- Guest from Rainfall Health

